



IDAMAN FERROUS FUMARATE TABLET 200 MG

PRODUCT DESCRIPTION

Round, reddish brown colored mottled tablet.

Each tablet contains: Ferrous Fumarate 200 mg.

PHARMACODYNAMICS

Iron is an essential constituent of the body, and is necessary for haemoglobin formation and for the oxidative processes of living tissues. Iron and iron salts should be given for the treatment or prophylaxis of iron deficiency anemias. Preparations of iron are administered by mouth, by intramuscular or intravenous injection. Soluble ferrous salts are most effective by mouth. Ferrous fumarate is an easily absorbed source of iron for replacement therapy. It is a salt of ferrous iron with an organic acid and is less irritant to the gastro-intestinal tract than salts with inorganic acids.

PHARMACOKINETICS

Absorption

In the acid conditions of the gastric contents, ferrous fumarate is dissociated and ferrous ions are liberated. These ions are absorbed in the proximal portion of the duodenum. The ferrous iron absorbed by the mucosal cells of the duodenum is oxidized to the ferric form, and this is bound to a protein to form ferritin.

Distribution

Ferritin in the mucosal cells releases iron into the blood, where it is bound to transferrin and passed into the iron stores - liver, spleen, and bone marrow. These stores are a reserve of iron for synthesis of hemoglobin, myoglobin, and iron containing enzymes.

Excretion

Iron is lost from the body through loss of cells in urine, faeces, hair, skin, sputum, nails, and mucosal cells, and through blood loss. Ferrous fumarate has the same pattern of absorption and excretion as dietary iron.

INDICATION

Idaman Ferrous Fumarate Tablet is indicated in the prophylaxis and treatment of iron deficiency resulting from inadequate diet, malabsorption, pregnancy, parasitic infection and/or blood loss. For prophylaxis during pregnancy, a combination of iron and folic acid is usually recommended.

RECOMMENDED DOSE

Adult

Prophylactic - Oral, 200 mg daily.

Therapeutic - Oral, 200 mg three or four times a day, the dosage being adjusted gradually as needed and tolerated

Children

Not recommended

This medicine is best taken on empty stomach, may be taken with food to lessen possibility of stomach upset.

ROUTE OF ADMINISTRATION

Oral

CONTRAINDICATIONS

Idaman Ferrous Fumarate Tablet is contraindicated in patients with:

- hemochromatosis;
- hemosiderosis;
- anemic conditions not due to iron deficiency;
- hypersensitivity to ferrous fumarate;
- paroxysmal nocturnal haemoglobinuria;
- active peptic ulcer;
- repeated blood transfusions;
- regional enteritis and ulcerative colitis.

WARNINGS AND PRECAUTIONS

Use of this medication should be carefully considered in patients with the following medical problems:

- hepatitis or hepatic function impairment;
- acute infectious kidney disease;
- intestinal tract inflammatory conditions such as enteritis colitis, diverticulitis, and ulcerative colitis;
- peptic ulcer;
- pancreatitis;
- rheumatoid arthritis;
- geriatrics: some geriatric patients may require a larger than usual daily ingestion of iron to correct a deficiency because their ability to absorb iron has been diminished by reduced gastric secretions and achlorhydria.

INTERACTIONS WITH OTHER MEDICAMENTS

Be alert of possible drug interactions when Idaman Ferrous Fumarate Tablet is taken together with the following:

- drugs used for treating infections, chloramphenicol, tetracyclines, ciprofloxacin, levofloxacin, moxifloxacin, ofloxacin, norfloxacin;
- medicines containing zinc, calcium or magnesium salts.
- levothyroxine (thyroxine), used for thyroid gland problems.
- medicines used for a disease known as 'Parkinson's disease' (i.e. levodopa, entacapone);
- methyl dopa, used for increase in blood pressure;
- penicillamine used in the treatment of rheumatoid arthritis;
- cholestyramine used to reduce cholesterol and fats in the blood;
- medicines for bone diseases (i.e. bisphosphonates);
- trientine, used for high copper levels in blood;
- dimercaprol, drug used for various metal poisonings;
- mycophenolate, used during organ transplant.

PREGNANCY AND LACTATION

Pregnancy

Idaman Ferrous Fumarate Tablet can be used during pregnancy if clinically indicated.

Lactation

No adverse effects of ferrous fumarate have been shown in breastfed infants of treated mothers. Idaman Ferrous Fumarate Tablet can be used during breast-feeding if clinically indicated.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

No known effects have been reported.

ADVERSE EFFECTS

The common adverse effects are:

Gastrointestinal disorders

Gastrointestinal irritation (nausea, epigastric pain, constipation, diarrhea, darkening of stools)

In the event of these adverse effects, it may be helpful to reduce the dose or switch to an alternative iron salt.

OVERDOSE AND TREATMENT

Each Idaman Ferrous Fumarate Tablet 200 mg contains approximately 66 mg of elemental iron.

Ingestion of 20 mg/kg elemental iron is potentially toxic and 200-250 mg/kg is potentially fatal. No single method of assessment is entirely satisfactory - clinical features as well as laboratory analysis must be taken into account. The serum iron taken at about 4 hours after ingestion is the best laboratory measure of severity.

Serum Iron	Severity
< 3 mg/L (55 micromol/L)	Mild toxicity
3-5 mg/L (55-90 micromol/L)	Moderate toxicity
> 5 mg/L (90 micromol/L)	Severe toxicity

Early signs and symptoms include nausea, vomiting, abdominal pain, haematemesis, melaena, rectal bleeding, lethargy, circulatory collapse, hyperglycaemia, metabolic acidosis and diarrhea. The vomit and stools may be grey or black. In mild cases, early features improve but in more serious cases, there may be evidence of hypoperfusion (cool peripheries and hypotension), metabolic acidosis and systemic toxicity. In serious cases, there can be recurrence of vomiting and gastrointestinal bleeding, 12 hours after ingestion. Shock can result from hypovolaemia or direct cardiotoxicity. Evidence of hepatocellular necrosis appears at this stage with jaundice, bleeding, hypoglycemia, encephalopathy and positive anion gap metabolic acidosis. Poor tissue perfusion may lead to renal failure. Rarely, gastric scarring causing stricture or pyloric stenosis (alone or in combination) may lead to partial or complete bowel obstruction 2-5 weeks after ingestion.

Treatment

Supportive and symptomatic measures include ensuring a clear airway, monitor cardiac rhythm, blood pressure and urine output, establishing intravenous access and administering sufficient fluids to ensure adequate hydration. Consider whole bowel irrigation. If metabolic acidosis persists despite correction of hypoxia and adequate fluid resuscitation, an initial dose of 50 mmol sodium bicarbonate may be given and repeated as necessary, for adults guided by arterial blood gas monitoring (aim

for a pH of 7.4). Consider the use of desferrioxamine if the patient is symptomatic (other than nausea), serum iron concentration is between 3-5 mg/L (55-90 micromol/L) and still rising. Haemodialysis does not remove iron effectively but should be considered on a supportive basis for acute renal failure as this will facilitate removal of the iron-desferrioxamine complex.

STORAGE CONDITION

Store below 30°C.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PACK SIZE

Blister pack of 10's (3 strips of 10 tablets)

REGISTRATION NO

MAL22016025XZ

KEEP MEDICINES OUT OF REACH OF CHILDREN JAUHI UBAT-UBATAN DARI KANAK-KANAK

For further information, please consult your doctor or your pharmacist.

Revision Number : 01

Revision Date : 04 Jun 2026

PRODUCT REGISTRATION HOLDER AND MANUFACTURER

INSPIRAPHARMA SDN. BHD. (200401023395)

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